

MERCY VALE REGIONAL MEDICAL CENTER — Hospital Medicine, 5 West Medical
5 Harbor Crest Way, Bridgehollow, PA 15211 • Main (412) 555-0143
CONFIDENTIAL — Protected Health Information

HOSPITAL DISCHARGE SUMMARY

Field

Detail

Patient

Korvin Merrow

MRN / FIN

KM-6427819 / KM-2026-051877

DOB / Age / Sex

02/18/1964 / 62 y / Male

Unit / Room

5 West Medical / 5W-318

Admission date

05/18/2026 (ED arrival 11:42)

Anticipated discharge

05/24/2026

Length of stay

7 days (HD1 05/18 → HD7 05/24)

Code status

Full Code (confirmed 05/18/2026)

Allergies

Lisinopril — cough (ACE-inhibitor intolerance)

Disposition

Home with services (see below)

Attending / Resident

Eliau Vossmer, MD (NPI 1528374960) / Ines Travyn, MD (PGY-2)

PCP

Talia Quenor, MD — Mercy Vale Primary Care, Harbor Crest

Note to the reader on the state of this chart. This summary is written from the full record through the morning of discharge. The last physician/care-team progress documentation in the chart is timestamped **05/23/2026 18:00 (HD6)**, at which point several transition items were still being finalized (final medication reconciliation, cardiorenal restart sequencing, the outpatient prednisone taper, home-health acceptance, durable medical equipment delivery, transportation, and the family supervision plan). Those items are carried below as **actions to be finalized at discharge / outstanding for follow-up** and are flagged explicitly so the PCP and outpatient team can close them without gaps. Where a source deliberately declined to assert a value — most importantly the pre-admission prednisone dose — this summary preserves that uncertainty rather than inventing a number.

DISCHARGE DIAGNOSES

Primary

Suspected urinary-source infection / sepsis physiology, treated empirically (clinical and laboratory response; culture/source not adjudicated as a single definitive cause).

Secondary / active during admission

Acute kidney injury on chronic kidney disease stage 3 — resolved to baseline by laboratory criteria.

Heart failure with reduced ejection fraction (HFrEF) and coronary artery disease s/p remote PCI/DES — chronic GDMT partially held during admission; staged restart in progress.

Type 2 diabetes mellitus — managed with basal insulin plus inpatient correctional insulin during acute illness; oral agents held.

Polymyalgia rheumatica on chronic prednisone — pre-admission dose/adherence not verifiable from inpatient sources; outpatient taper to be coordinated with rheumatology and endocrinology.

Functional decline and deconditioning with high fall risk, and altered mental status relative to baseline (improving) — multifactorial.

Chronic comorbidities carried (stable): essential hypertension; mixed hyperlipidemia; diabetic peripheral neuropathy; obstructive sleep apnea (CPAP, variable adherence); anemia of CKD; steroid-associated osteoporosis/osteopenia; class I obesity (BMI 30.6); chronic GERD; chronic constipation tendency.

REASON FOR ADMISSION / PRESENTING PROBLEM

Mr. Merrow is a 62-year-old man with HFrEF, CAD s/p remote mid-LAD drug-eluting stent (2018), CKD stage 3, type 2 diabetes, polymyalgia rheumatica on chronic prednisone of uncertain current dose, OSA, and related chronic illness, brought to the ED by his wife on **05/18/2026** after roughly **three weeks of progressive decline** — reduced appetite and fluid intake, falling stamina, increasing dependence for usual tasks, and intermittent **evening confusion** described by family as “not his normal.” The proximate trigger was a **near-fall at home on 05/17/2026** when he stood from a kitchen chair (no head strike, no loss of consciousness; his wife steadied him). He endorsed possible urinary burning/frequency “a few days back” but was an inconsistent historian. He denied chest pain, rest dyspnea, or focal neurologic symptoms.

ED findings (HD1): low-grade fever 100.3 °F, mild sinus tachycardia (HR 104), low-normal BP 102/62, SpO2 95% on room air, dry mucous membranes and reduced skin turgor, oriented to person/place but missing the date. Labs: **creatinine 2.62 mg/dL** (baseline 1.6–1.8), BUN 48, **potassium 5.1**, **WBC 15.6** with left shift, glucose 180–260; **urinalysis with pyuria, positive leukocyte esterase, and bacteriuria**. ECG and chest imaging without acute findings; no troponin-defined ACS. Urine and blood cultures were drawn **before** the first antibiotic dose. The ED initiated renally-dosed empiric antibiotics, cautious crystalloid (small boluses given HFrEF), held the cardiorenal/antihypertensive regimen, transitioned glucose management to insulin, and admitted to Hospital Medicine (Dr. Vossmere) on 5 West. The team framed this as suspected urinary-source sepsis with AKI-on-CKD and dehydration, while explicitly keeping the differential broad (volume depletion, electrolytes, chronic steroid exposure, cardiorenal medication burden, deconditioning).

HOSPITAL COURSE (the way it actually unfolded)

Mr. Merrow improved steadily over six documented hospital days along parallel tracks — infection markers, renal function, hemodynamics, glycemia, intake, cognition, and mobility — though not every problem returned fully to baseline, and the most consequential medication decisions were deliberately staged rather than forced to a single point.

Objective
trajectory,
HD1 →
HD6

Date (HD)

Creatinine

BUN

K⁺

WBC

Systolic BP

Glucose

Meal intake

05/18 (HD1)

2.62

48

5.1

15.6

96-110

180-260

25-40% (poor)

05/19 (HD2)

2.38

45

4.9

13.8

100–118

170–250

40–55%

05/20 (HD3)

2.12

39

4.7

11.9

106–124

160–240

55–70%

05/21 (HD4)

1.98

35

4.6

10.8

110–128

160–270

50–75%

05/22 (HD5)

1.86

31

4.5

9.9

112–128

150–260

70–85%

05/23 (HD6)

1.80

29

4.4

9.4

114–130

140–250

75–90%

Baseline comparators (outpatient): creatinine 1.6–1.8 mg/dL, eGFR 40–50 mL/min/1.73 m², hemoglobin 10.5–11.5 g/dL, A1c 7.6–8.2%, dry weight ~97 kg.

Infection

Empiric **ceftriaxone 1 g IV q24h** was started in the ED after cultures and continued HD1–HD3. Fever defervesced early (Tmax 38.2 °C HD1 → afebrile from HD3 on) and the WBC fell steadily (15.6 → 9.4). With clinical response, therapy was **stepped down to oral cefpodoxime 200 mg PO BID on HD4** and continued HD4–HD6. The HD6 MAR flags that **completion versus continuation of the antibiotic course was to be reconciled at discharge** — this should be confirmed on the final medication list and communicated to the patient/family and PCP. Throughout, the team was careful not to treat “urinary-source sepsis” as a sufficient explanation for every feature of the presentation (residual weakness and cognitive slowing in particular).

AKI on CKD

AKI-on-CKD was judged **multifactorial with a dominant prerenal contribution** (poor intake and infection-related hemodynamics) layered on chronic RAAS/neprilysin therapy, mineralocorticoid antagonism, and SGLT2 inhibition. Management was **cautious volume repletion** mindful of HFrEF — a single 500 mL normal-saline bolus then a downtitrated infusion (75 → 50 → 30 mL/hr), off continuous fluids and onto a saline lock by HD4 — with daily BMPs, strict I/O, and daily weights; NSAIDs avoided. Creatinine improved each day (2.62 → 1.80) and potassium normalized (5.1 → 4.4). **Nephrology (Dr. Solthar) documented on HD6 that the AKI had resolved to the patient’s baseline by laboratory criteria**, while emphasizing that lab recovery achieved on a monitored floor is not the same as restored physiologic reserve at home.

HFrEF / CAD and the cardiorenal-restart question (central medication issue)

On admission, the hemodynamic- and renal-sensitive HFrEF agents were held: **sacubitril/valsartan, spironolactone, furosemide, and empagliflozin**. CAD secondary prevention — **aspirin 81 mg daily and atorvastatin 40 mg nightly** — was continued throughout. **Carvedilol** was handled by shift-level hemodynamics: held HD1–HD2 (SBP 98–104, symptomatic), half-day HD3, then **resumed cautiously at 12.5 mg BID from HD4 through HD6** as systolics improved to 116–124.

Cardiology and Nephrology were both consulted on HD2 and filed **dated, deliberately non-collapsed positions** that the primary team preserved rather than reconciling prematurely:

Cardiology (Dr. Caldrene): preserve HFREF/CAD protective therapy; prolonged loss of GDMT raises decompensation and 30-day readmission risk. Favored cautious **early reintroduction of the beta-blocker** (hence carvedilol resumed) and urged that the team begin **planning a staged restart sequence now** — selecting agents, doses, and parameter triggers — rather than waiting for one fully-recovered moment.

Nephrology (Dr. Solthar): renal recovery, potassium, BP reserve, and intake all remained fragile; a creatinine still above the outpatient anchor “after only a few days of holds is not restored reserve.” Restart should be **staged, single-agent, and parameter-gated** (creatinine/eGFR, potassium, BP margin, intake/weight), not simultaneous and not calendar-driven.

No agent among sacubitril/valsartan, spironolactone, furosemide, or empagliflozin was restarted during the admission. As of the last note, the staged sequence had been outlined but no final restart order set had been written; restart timing is to be coordinated between Cardiology, Nephrology, and the PCP after discharge. Cardiology explicitly did **not** declare unilateral restart safe at the moment of discharge. Telemetry showed sinus rhythm without significant ectopy; no ischemic features and no advanced imaging indicated this admission; sublingual nitroglycerin was available but never required.

Diabetes

Oral agents (**metformin ER** and **empagliflozin**) were held for AKI/acute illness. **Basal insulin glargine** was continued with inpatient adjustment — reduced to 12 units on HD1 during poor intake and **re-titrated back to the home 18 units nightly by HD6** — alongside a **correctional insulin lispro** scale that tapered as glucose improved (POC glucose 232 → 182). A1c baseline is 7.6–8.2%. Endocrinology recommended against starting any new antidiabetic agent in hospital and deferred outpatient regimen review to the PCP and endocrinology.

Polymyalgia rheumatica and the prednisone-provenance problem

This was a recurring theme and is deliberately left **unresolved on a specific pre-admission dose**, because no source could verify what he was actually taking:

Inpatient medication reconciliation (Marlott, PharmD): 18 of 19 home medications verified across patient, family, pharmacy fills, and the PCP list; **prednisone was the single unverified, highest-priority item**, with *no numeric current dose asserted*.

Pharmacy refill history (Harbor Crest): prednisone dispensed at two strengths over six months with **changing taper-style directions** — 10 mg daily (Nov–Jan) → 5 mg daily (Feb) → 5 mg ½-tablet (2.5 mg) daily (Mar) → 5 mg “taper per provider instructions” (Apr), most recent fill **05/06/2026 at 5 mg with taper-style directions**. The pharmacy explicitly cautioned that fill directions do not establish what was consumed and that older bottles with superseded directions may remain in circulation at home.

Outpatient rheumatology (Dr. Halvek, Riverbend): confirmed a multi-year PMR course and a **standing plan of cautious slow taper if symptoms remain controlled, avoiding abrupt discontinuation, with instructions to contact rheumatology/PCP if PMR or systemic symptoms flare**. Halvek deliberately did **not** specify a number, pointing to the verified med rec, fill history, and any active e-prescription as the operative records.

Family (Mara): the bottle directions and Korvin’s actual use “may not match,” and he could not reliably reconstruct recent dosing.

Interim inpatient steroid coverage was given daily HD1–HD6 (non-abrupt), and **the regimen was not stopped**.

Endocrinology (Dr. Veylorn) was consulted HD4 and framed chronic cumulative steroid exposure as a **real longitudinal risk substrate** (bone health, glycemic burden, infection vulnerability, and the possibility of relative adrenal insufficiency under stress) while stating clearly that **adrenal insufficiency is NOT established** — the HPA axis

cannot be characterized during acute illness and requires non-stressed outpatient testing. Endocrinology and the primary team held these two views simultaneously: the steroid history matters and warrants safe handling and outpatient follow-through, but it should **not** be assumed to be the sole explanation for the residual weakness, and **no new long-term taper dose should be invented from the inpatient record**. A structured taper is to be finalized at discharge in consultation with rheumatology, with the long-term plan owned in the ambulatory setting.

Functional status, cognition, and medication-management capacity

These became a distinct readiness axis, separate from the medical trajectory.

PT (Volkos, DPT; HD3 → HD5 reassessment): improved from 10–15 ft with a rolling walker and standby assist (HD3) to **100–150 ft with a rolling walker and supervision (HD5)**; sit-to-stand improved to supervision. **Morse Fall Scale 65 (high risk)** persisted. PT recommended a **rolling walker (not the baseline cane)**, bedside commode initially, bathroom grab bars, removal of loose rugs, and **supervised mobility for the first 5–7 days**; endurance/orthostatic findings did **not** support cane-only or unsupervised ambulation at discharge.

OT (Sayre, OTR/L; HD3 → HD5): ADLs improved to supervision, but **medication-management deficits were reproducible across sessions** — on his 19-item regimen he made recurrent errors with BID timing and PRN-versus-scheduled distinction and could not describe the prednisone taper. OT recommended a **pre-filled weekly pill organizer (not his sole responsibility)**, **teach-back with family for any new/changed medication**, **daily family check-ins at medication times for 5–7 days (verifying doses were actually taken)**, and outpatient OT if no progress by ~30 days.

Nursing / cognition: intake rose from 25–40% to 75–90%; evening confusion was frequent early (HD1–HD4; e.g., trying to get up “to check on the dog,” asking if he was at the rheumatology clinic) and became **less prominent by HD5–HD6**, though he remained slow on multi-step medication questions through HD6. **Gabapentin** was held HD1–HD2 and reduced to 100 mg HD3–HD4 for mentation/fall-risk concerns, then **returned to the home 300 mg nightly on HD5–HD6** as cognition cleared. OSA: CPAP available in hospital; not a driver of this presentation.

Family involvement and disposition planning

Mara Merrow (wife, healthcare proxy, primary caregiver) was present daily and engaged; daughter Lenora Merrow-Halsey provides intermittent support (weekend daytime and transportation; **not reliably available weekday mornings**). A formal **care conference (HD5, 05/22)** with the hospitalist team, Case Management (Priya Ostroff, RN, CCM), and Social Work (Devra Aimes, LCSW) set a working discharge target of 05/24/2026. Her consistent, collaborative position — *“Better than Monday is not the same as ready to manage the next steps without support”* — generated five specific requests that map directly onto this handoff: (1) a **final medication review with family present** before leaving; (2) the **heart and kidney medications explained together** (held vs. restarting vs. conditional on labs); (3) **prednisone instructions reconciled with Dr. Halvek’s history**; (4) **explicit supervision expectations** (overnight, medication times, ambulation) by family member and window; and (5) **specific warning signs** with a clear contact. Case Management initiated home-health screening (candidate agency **Keystone HomeCare Services, (412) 555-0631**), a DME plan (rolling walker, bedside commode, grab bars), transportation options, and first-fill coordination with **Harbor Crest Community Pharmacy**.

CONDITION AT DISCHARGE

Afebrile and hemodynamically stable (SBP 114–130). Creatinine **1.80 mg/dL** (upper end of baseline), potassium 4.4, WBC 9.4, POC glucose 140–250. Eating 75–90% of meals. **Ambulates short-to-moderate distances with a rolling walker and supervision; remains a high fall risk (Morse 65)**. Alert and oriented with prompting; **residual slowing on**

multi-step/medication tasks that has improved but not fully returned to baseline. Improved overall and appropriate for discharge home **with the services, supervision, and follow-up below**.

ACTIVE PROBLEMS AND WHERE THINGS STAND

Suspected urinary-source infection — responding. Completed IV ceftriaxone → oral cefpodoxime. **Confirm completion vs. remaining days of the oral course on the final discharge list** (open per HD6 MAR) and communicate the stop date to the patient/family and PCP.

AKI on CKD stage 3 — resolved to baseline (Cr 1.80). Underlying CKD stage 3 unchanged (eGFR 40–50). Avoid NSAIDs; renal-dose any new prescription. **Early outpatient BMP** to confirm renal/potassium stability, especially if any cardiorenal agent is restarted.

HFrEF / CAD — GDMT partially held; staged restart pending. Continue **aspirin 81 mg daily and atorvastatin 40 mg nightly**. Carvedilol 12.5 mg BID was reintroduced and tolerated inpatient. **Sacubitril/valsartan, spironolactone, furosemide, and empagliflozin remain HELD** at discharge; restart to be **staged, single-agent, and parameter-gated** (creatinine/eGFR, potassium, BP margin, intake/weight) and **coordinated between Cardiology, Nephrology, and PCP** — not restarted simultaneously. No agent was restarted in hospital.

Type 2 diabetes. Insulin glargine 18 units nightly (home dose, re-achieved inpatient) continued. **Metformin ER and empagliflozin remain held** pending cardiorenal/renal clearance; empagliflozin restart is tied to the HFrEF sequencing. No new agent started. Outpatient regimen review with PCP/endocrinology (A1c baseline 7.6–8.2%).

PMR / chronic prednisone — provenance unresolved; do not assume a number. Steroids were continued (non-abrupt) in hospital; **do not stop abruptly. Finalize a structured taper at discharge in consultation with Dr. Halvek**, with definitive taper management in the ambulatory setting. **Adrenal insufficiency not established**; outpatient HPA-axis testing under non-stressed conditions if clinically indicated.

Functional decline / high fall risk. Rolling walker and supervised mobility for the first 5–7 days; bedside commode initially; bathroom grab bars; remove loose rugs; verify front-step handrail. Home vs. outpatient PT/OT per the discharge functional plan.

Medication-management capacity. Pre-filled weekly pill organizer with family oversight; teach-back for any new/changed medication; **daily family verification at medication times for 5–7 days**; do not make Korvin solely responsible for new/changed medications during the early window.

Chronic/stable, continued: pantoprazole 40 mg daily (GERD); gabapentin 300 mg nightly (neuropathy; monitor sedation/fall risk); ferrous sulfate 325 mg QOD (anemia of CKD); **alendronate 70 mg weekly** (not given inpatient — **reconcile/resume outpatient**); calcium carbonate/vitamin D 600 mg/400 IU BID; PEG 3350 and senna PRN (bowel regimen); nitroglycerin 0.4 mg SL PRN (rescue). OSA — continue home CPAP; reinforce adherence.

DISCHARGE MEDICATIONS
— STATUS AT A GLANCE

The final reconciled list/prescriptions are to be completed by the discharging team and pharmacy on 05/24; this table reflects the medication status carried from the inpatient record and should be confirmed against that final reconciliation.

Medication

Status at discharge

Notes

Aspirin 81 mg PO daily

Continue

CAD secondary prevention

Atorvastatin 40 mg PO nightly

Continue

CAD / hyperlipidemia

Carvedilol 12.5 mg PO BID

Continue

Reintroduced HD4, tolerated; monitor BP/HR

Sacubitril/valsartan 24/26 mg PO BID

HELD

Staged, parameter-gated restart (Cards/Nephro/PCP)

Spirolactone 25 mg PO daily

HELD

Watch K⁺; restart per sequencing

Furosemide 40 mg PO daily

HELD

Restart per volume status / sequencing

Empagliflozin 10 mg PO daily

HELD

Restart tied to renal/HFrEF sequencing

Metformin ER 500 mg PO BID

HELD

Restart after renal clearance

Insulin glargine 18 units SC nightly

Continue

Home dose; adjust per outpatient glucose

Prednisone (PMR)

Continue — taper to be set with rheumatology

No abrupt stop; no verified pre-admission dose asserted

Gabapentin 300 mg PO nightly

Continue

Monitor sedation / fall risk

Pantoprazole 40 mg PO daily

Continue

GERD

Ferrous sulfate 325 mg PO QOD

Continue

Anemia of CKD

Alendronate 70 mg PO weekly

Resume outpatient

Not given inpatient; reconcile

Calcium carbonate/vit D 600 mg/400 IU PO BID

Continue

Bone health

PEG 3350 17 g PO daily PRN; Senna 8.6 mg PO nightly PRN

Continue PRN

Bowel regimen

Nitroglycerin 0.4 mg SL PRN chest pain

Continue PRN

Rescue; not used inpatient

Cefpodoxime 200 mg PO BID

Confirm completion/stop date

Step-down from IV ceftriaxone; reconcile at discharge

Medication cautions for the outpatient team: avoid NSAIDs (renal); potassium-affecting agents (sacubitril/valsartan, spironolactone) were held and should be restarted only with lab follow-up; do not stop

prednisone abruptly; reconcile the prednisone dose/taper with Dr. Halvek rather than inferring from bottles or fill directions.

FOLLOW-UP PLAN

Service

Clinician

Target window

Purpose

Primary care

Talia Quenor, MD (Harbor Crest)

Within 7 days

Post-discharge reconciliation, BMP/electrolytes, integrate specialty plans, oversee cardiorenal restart

Cardiology

Maris Caldrane, MD

Within 2 weeks

GDMT restart titration, BP/orthostatic assessment, final ambulatory regimen

Nephrology

Iven Solthar, MD

Within 1-2 weeks

Renal/electrolyte monitoring; gate cardiorenal restarts (sooner if an agent is restarted near discharge)

Endocrinology

Nerea Veylorn, MD

Within 2-4 weeks

Steroid/glycemic management; consider HPA-axis testing; diabetes regimen review

Rheumatology

Soren Halvek, MD (Riverbend)

Within 2–4 weeks

Finalize/confirm the prednisone taper

Labs: early outpatient **BMP** (creatinine, potassium) to confirm stability and to safely guide any cardiorenal restart; coordinate through PCP. Outpatient draws available at Mercy Vale Outpatient Laboratory — Harbor Crest.

Cross-coverage coordination the outpatient team should expect to close:

Cardiorenal restart sequencing (sacubitril/valsartan, spironolactone, furosemide, empagliflozin): single-agent, parameter-gated, coordinated between Cardiology and Nephrology with PCP oversight and lab follow-up after each change.

Prednisone taper finalized with Rheumatology; **no numeric pre-admission dose is asserted in this record.**

Diabetes oral agents (metformin/empagliflozin) restart tied to renal clearance and the HFrEF sequence.

Alendronate resumption reconciled outpatient.

HOME SERVICES, EQUIPMENT, AND SUPERVISION

Disposition: home (single-story residence, bedroom on main level, one main-level half-bath, three exterior entry steps — **verify front-step handrail**; remove loose rugs from walking paths). Address on file: 1842 Larchmere Lane, Bridgehollow, PA.

Home health nursing — candidate agency **Keystone HomeCare Services, (412) 555-0631** for early post-discharge medication review and monitoring. *Acceptance/authorization was pending at the last note — confirm before or at discharge.*

Therapy — home vs. outpatient PT/OT per the discharge functional plan; outpatient OT for medication-management retraining if no progress by ~30 days.

Durable medical equipment — **rolling walker** (primary device; not the baseline cane), **bedside commode** initially, **bathroom grab bars**. *Delivery was pending confirmation — confirm at discharge.*

Supervision (first 5–7 days) — supervised mobility; **daily family check-ins at medication times with verification that doses were taken**; pre-filled weekly pill organizer maintained by family. Define who covers which window (note daughter's weekday-morning limitation).

Transportation — family vehicle and/or insurance-linked non-emergency medical transport (Keystone Senior Supplement Plan G) / community senior van; confirm for the +7-day PCP visit.

Pharmacy — Harbor Crest Community Pharmacy, (412) 555-0490; coordinate first-fill timing for any new/changed medications.

Family education at discharge — conduct the final medication review with Mara (and Lenora by phone) present; teach-back; explain held vs. continued vs. restarting medications and the prednisone plan; review warning signs and contacts.

Key contacts: Wife/proxy Mara Merrow (412) 555-0229; daughter Lenora Merrow-Halsey (412) 555-0231; Case Management/Social Work (412) 555-0162.

RETURN PRECAUTIONS / WARNING SIGNS

Return to the Emergency Department or call (412) 555-0150 for: **fever or chills; recurrent or worsening weakness, lightheadedness, or another near-fall/fall; new or worsening confusion; chest pain or shortness of breath / orthopnea; weight gain or increasing swelling (HFrEF decompensation); reduced oral intake or signs of dehydration; decreased urination; or symptoms of a PMR flare or systemic illness** (contact rheumatology/PCP). Given chronic steroid exposure, if he develops a **febrile illness, persistent vomiting, or significant dehydration**, contact his physicians promptly for stress-dosing guidance **rather than stopping or self-adjusting the prednisone**. Review angina-equivalent symptoms given the prior LAD stent.

Prepared from the complete record (ED triage through HD6, 05/23/2026 18:00) for discharge on 05/24/2026. Items marked pending/outstanding reflect the state of the chart at the last documented timestamp and are to be finalized by the discharging team at the time of the discharge order.

Attending of record: Elian Vossmere, MD — Hospital Medicine, Mercy Vale Regional Medical Center (NPI 1528374960) **Resident:** Ines Travyn, MD (PGY-2)